

her

First Health Insurance

Here's what you actually need to know
before buying.—————→

A long-term **Financial Safety System** for

Women in India

Women are insured, but not protected

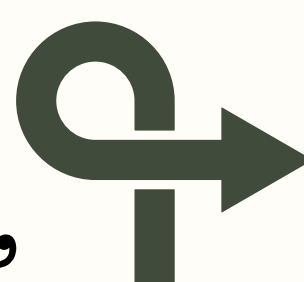
They have:

- Employer coverage
- Family/spouse policy

But here's the flaw:

These are dependent covers, not owned assets.

The moment life shifts,



job loss, career break, or divorce coverage disappears instantly.

Healthcare costs don't just rise, it **compound**.

In India:

- Medical inflation ~14% annually
- Out-of-pocket expenses push ~6 crore people into poverty every year

Health insurance is not optional anymore. It's a financial
buffer against uncertainty.



Women-Specific Risks

Women's healthcare is structurally different, across a lifetime, costs include:

- Hormonal conditions (PCOS, thyroid)
- Reproductive health (maternity, endometriosis)
- Higher cancer risks (breast, cervical)
- Longer life expectancy → longer coverage needed

Yet most people still buy **“generic”** policies.

What dependency on employer cover looks like

A 29-year-old professional lost her job.

Within weeks:

- No health cover
- Diagnosed with thyroid condition
- Paid ₹32,000 out-of-pocket

The issue wasn't income.

It was lack of **independent cover.**



The Cost Illusion

“Cheap premium” is the most expensive mistake

Example pattern:

- ₹3 lakh cover feels sufficient
- Major illness costs ₹8–10 lakh
- Policy exhausts in first phase of treatment

The gap is funded by:
Savings, loans, or family.

Accurate Coverage

How much cover actually makes sense

In metro cities:

- Early career → ₹5–10 lakh
- Mid career → ₹10–15 lakh
- Add super top-up → additional ₹10–20 lakh

Why?

Because future upgrades become:

More expensive + restricted after diagnosis.

Policy Design Thinking

One policy is rarely enough

Smart structure looks like:

- Base policy → covers hospitalisation
- Super top-up → protects against large bills
- Critical illness → provides liquidity

This is not over-insurance.

This is **layered risk management.**

Family Floaters

Why shared policies fail silently
Family floater = one shared pool.

Real scenario:

- One surgery → ₹2 lakh
- Another hospitalisation → ₹1 lakh

Remaining cover becomes inadequate for the year.

Solution:

Have individual cover + floater (optional)

Fine Print

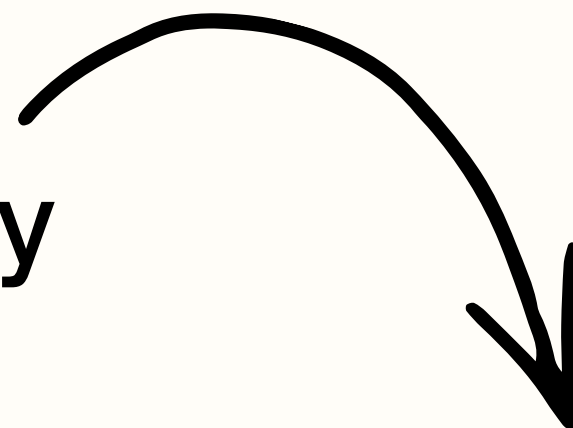
Where most claims actually get reduced
Not rejection, but reduction.

Because of:

- Room rent limits
- Sub-limits on procedures
- Co-pay clauses

Example:

₹2 lakh surgery



only ₹50,000 reimbursed due to sub-limit.

Terms That Change Everything

If you understand these, you won't be mis-sold

- Sum Assured → Your real protection ceiling
- Waiting Period → When coverage actually starts
- Restoration Benefit → Refill after exhaustion
- No Claim Bonus → Free increase in cover

These define your actual policy value.



Rider Strategy

When add-ons actually work

Example:

- Hospital cash rider → ₹1,500/day
- 8-day hospitalisation → ₹12,000 payout

This covered income loss — not hospital bills.

Insight:

Riders solve specific problems, not everything.

Buying Framework

A Step-wise way to choose your first policy

- Step 1.** Assess health + family history
- Step 2.** Factor life plans (pregnancy, career gaps)
- Step 3.** Compare 3–5 insurers
- Step 4.** Read exclusions carefully
- Step 5.** Choose higher cover early
- Step 6.** Set renewal discipline

This is a financial decision — not a quick purchase.

Claim Reality

Cashless vs Reimbursement, what actually happens

Cashless :

- Faster
- Less financial stress
- Works only in network hospitals

Reimbursement :

- You pay upfront
- Claim later
- Delays possible

Choosing the right insurer reduces friction here.

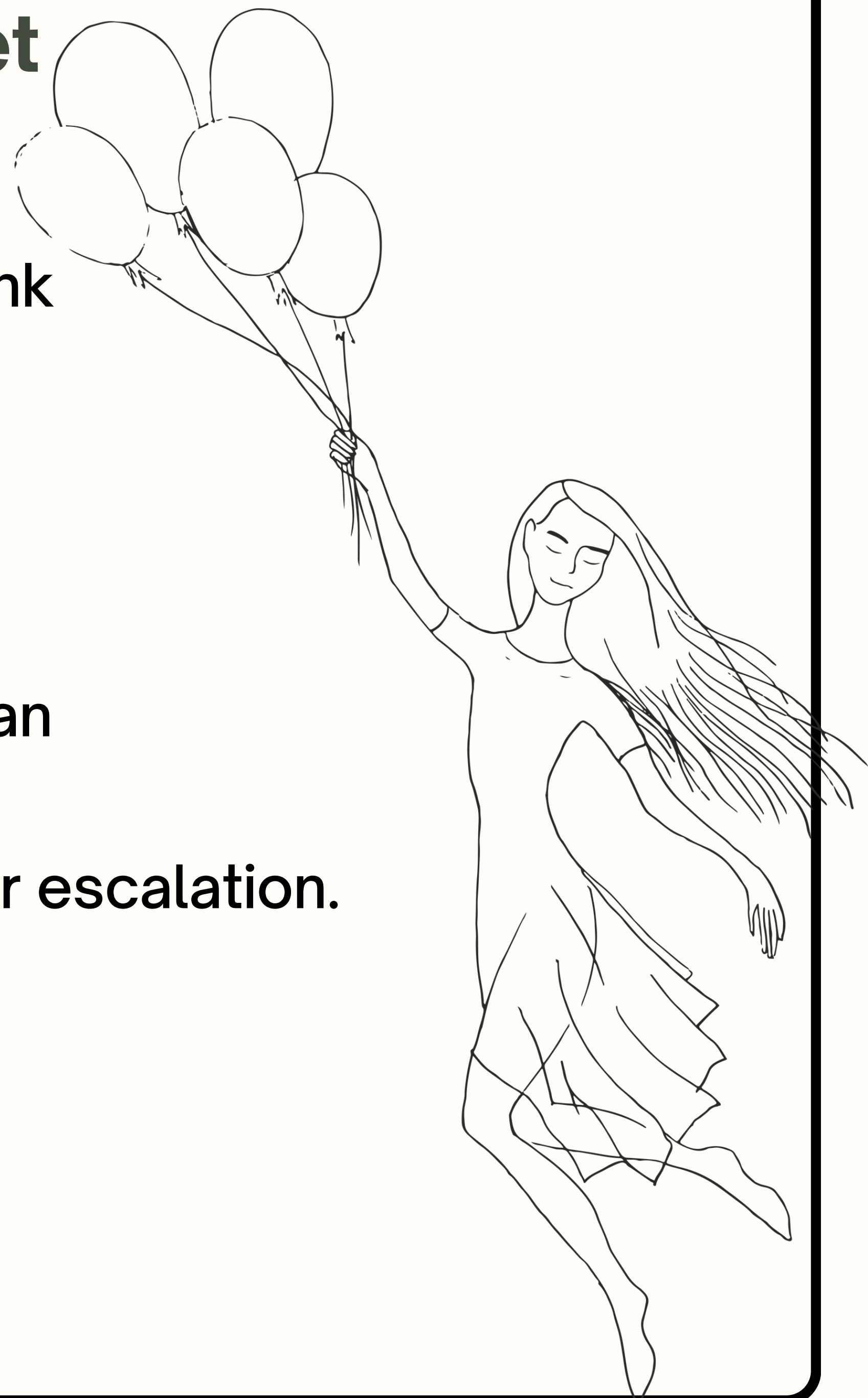
Rights Most Women forget

You have more control than you think

If insurer delays or rejects unfairly:

- Raise complaint with insurer
- Escalate to IRDAI
- Approach Insurance Ombudsman

Many claims get resolved only after escalation.



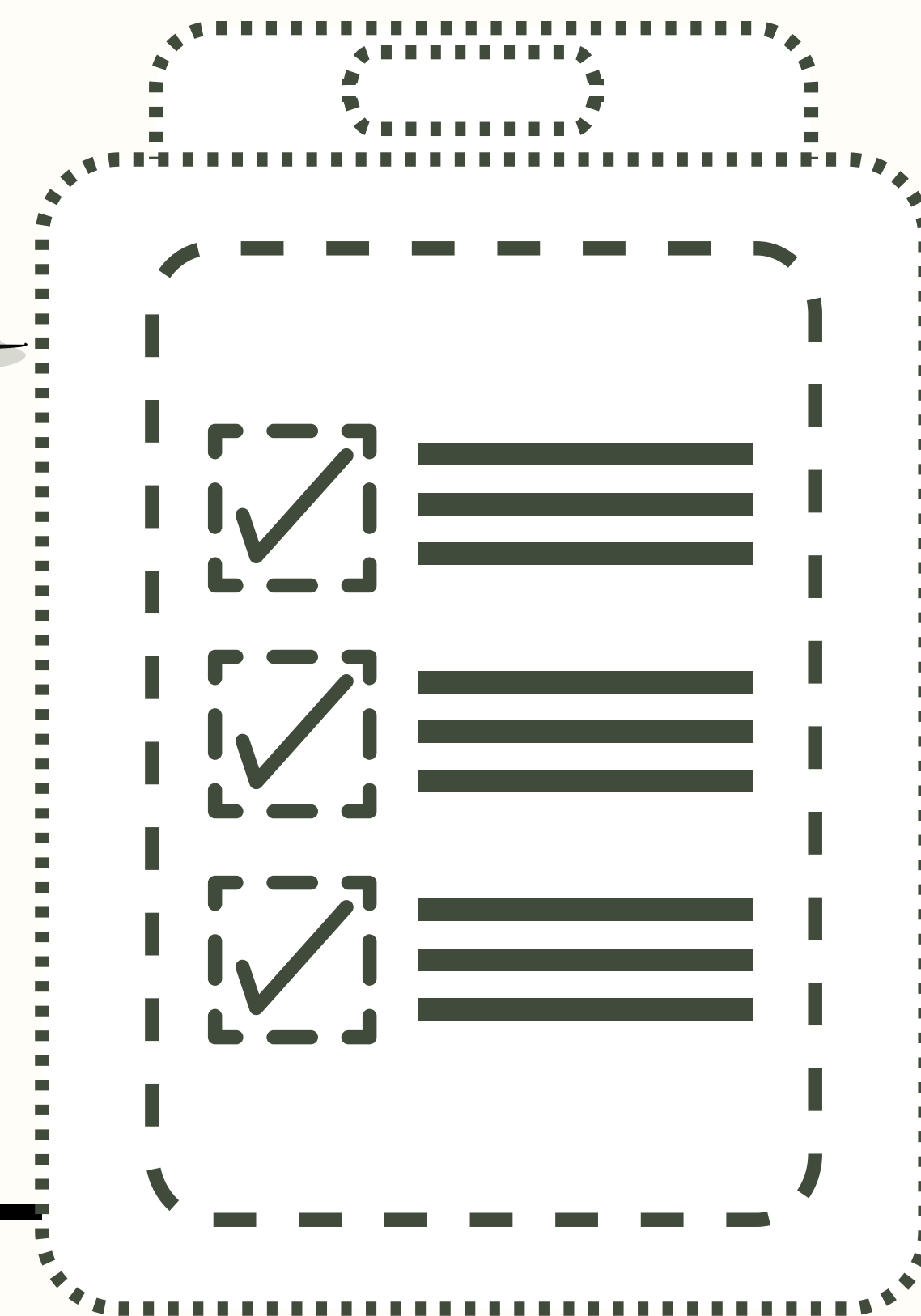
Golden Rules

If you remember nothing else, remember this

- Don't depend only on employer cover
- Don't optimise for lowest premium
- Don't ignore policy wording
- Don't delay buying

And most importantly:

Buy before your health history changes.



Health insurance is not about hospitals

It's about:

- Keeping your savings intact
- Making independent decisions
- Handling uncertainty without panic

The right policy doesn't feel important today.
It becomes critical on the worst day.





MoneyMati

Own your wallet, own your life.

@moneymati

